

MEDICAL STOP LOSS CAPTIVES

Structure, Strategy, and Market Evolution



EXECUTIVE SUMMARY

Medical stop loss captives are no longer niche they are becoming the preferred strategy for sophisticated employers. Employers are demanding control, participation, and alignment not just protection.

Captives deliver all three.

This paper outlines how captives work, why they outperform traditional models, and where the market is going.

WHAT IS A MEDICAL STOP LOSS CAPTIVE

A captive is a risk-sharing structure where employers:

- Retain a layer of risk
- Pool exposure
- Access reinsurance
- Participate in underwriting outcomes

Employers move from premium payers to risk participants.

THE LIMITATIONS OF TRADITIONAL STOP LOSS

Traditional stop loss is protective not strategic. Carriers price conservatively, retain profits, and limit transparency. Employers take operational responsibility without financial upside.

This misalignment creates inefficiency.

CORE STRUCTURE

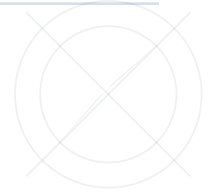
Layer 1: Employer Retention (e.g., \$0-\$100K)

Layer 2: Captive Layer (e.g., \$100K-\$300K pooled)

Layer 3: Carrier/Reinsurance (catastrophic protection)

Surplus remains within the captive and returns to the participants.

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WHY CAPTIVES OUTPERFORM

- Risk pooling reduces volatility
- Employers participate in underwriting profit
- Reduced carrier margin
- Aligned incentives across stakeholders

UNDERWRITING EVOLUTION

Captive underwriting is forward-looking and dynamic. It incorporates population health, engagement, and cost containment. Underwriting is no longer just predicting risk it is shaping it.

COST CONTAINMENT MULTIPLIER

Cost containment amplifies captive performance.

Lower claims - improved loss ratios - surplus generation - lower net cost.

This creates compounding financial benefit.

MARKET TRENDS

- Mid-market adoption accelerating
- MGU platforms expanding
- Clinical integration increasing
- Data-driven underwriting emerging

ZENITH PERSPECTIVE

Most captives are structured correctly but a few are optimized. Zenith focuses on underwriting precision, clinical integration, and execution. Captives are not a product; they are a platform.

CONCLUSION

Captives represent a shift from passive buying to active risk management. Employers gain control, reduce costs, and align incentives. Those who adopt early will outperform.